

Activity 8 — Building the Corrective Action Plan

Meridian Health — from chosen solutions to an auditable plan

Topic	Topic 2
Objective	A4 · K9 — Develop corrective action plans for shortfalls identified.
Duration	45 minutes
Grouping	Teams of 3-5
Tools	ChatGPT / Copilot / Gemini, Pinboard
Ed-tool	Pinboard — https://alfredang.github.io/pinboard/

The Scenario

The Clinical Director has approved your four solutions. She now asks the question that separates a good analysis from a delivered result:

"Who is doing what, by when, with what money, and how will I know it worked?"

She also adds a warning from experience: "The last improvement project we ran had a beautiful slide deck and no owner. Six months later nothing had changed and everyone assumed someone else was doing it."

Your team must convert four approved solutions into a corrective action plan that would survive an internal audit — and that a colleague could execute without you in the room.

The Evidence

Corrective Action Plan — mandatory components (K9)

Component	Why it exists	Failure if missing
Root cause addressed	Ties the action to the diagnosis	Solution drifts to a symptom
Corrective action	The specific thing being done	Vague intent, no execution
Owner (named person)	Single point of accountability	Everyone assumes someone else
Timeline / milestone	Makes progress checkable	Slips indefinitely, unnoticed
Resources required	Budget, people, tools committed	Stalls at first resource conflict
Success measure + baseline	Defines what 'worked' means	Cannot evaluate; endless debate
Risk and mitigation	Anticipates what breaks it	Surprised by predictable failure
Review checkpoint	Forces a decision to continue/stop	Runs on past the point of failure

Discussion Questions

1. Complete all eight components for each of your four actions. Which component did your team find hardest to fill honestly, and what does that reveal?
2. Every owner must be a named role, not a department. Why does 'Operations' fail as an owner where 'Centre Operations Manager' succeeds?

3. Check your success measures against the baseline in your original problem statement. Can each one actually be measured with data Meridian already collects? Which cannot?
4. The AI flagged dependencies between actions. Which two of your actions must be sequenced, and what breaks if you run them in parallel?
5. Identify the biggest risk across your whole plan. Is your mitigation a real action with an owner, or a hope?

Step-by-Step

6. List your four approved solutions from Activity 7 across the top of a flip chart.
7. Draw the eight CAP components down the side as rows.
8. Fill in 'Root cause addressed' for all four FIRST. If any solution cannot be traced to your diagnosed root cause, challenge whether it belongs in the plan.
9. Assign a NAMED ROLE as owner for each action. Reject any department name. One owner per action — no co-owners.
10. Add timelines with a start, a milestone and a completion date. Use week numbers, not 'Q3'.
11. Cost each action and check the four together fit inside S\$250,000.
12. Write each success measure as metric + baseline + target. Cross-check the baseline against your Activity 1 problem statement.
13. Test each measure: can Meridian measure this with data it already collects? If not, either change the measure or add 'establish measurement' as a task.
14. Add the key risk and a mitigation that is itself an action with an owner — not a hope.
15. Set a review checkpoint for each action and state the DECISION to be made at it.
16. Run the GenAI corrective action plan prompt from the slide.
17. Act on the AI's three flags: sequence the dependent actions, rebalance any overloaded owner, and fix any unmeasurable success measure.
18. Post the completed plan to <https://alfredang.github.io/pinboard/> and present one row in full to the room.

The GenAI Prompt

You are an execution-focused planning assistant. Convert each approved solution into a corrective action plan row. Be concrete and brief – no theory.

For EACH solution, produce exactly these fields:

- Root cause addressed (link back to the diagnosis)
- Corrective action (one specific sentence)
- Owner (a job title, not a department)
- Timeline (start, key milestone, completion)
- Resources (budget in S\$, people, tools)
- Success measure (metric + baseline + target)
- Key risk + mitigation
- Review checkpoint (date and the decision to be made at it)

Then flag, in a separate section:

1. Any two actions that depend on each other and must be sequenced
2. Any owner who appears on more than two actions (overload risk)

3. Any success measure that cannot actually be measured with data the organisation already has

Approved solutions: [paste your four here]

Root cause: 44% phlebotomist turnover leaving 40% of the 07:00 roster untrained, causing 14-min draws vs 6-min, 31% appointment overruns and 23% of samples missing the 09:45 courier.

Budget: S\$250,000 over 12 months.

Trainer Debrief

Expected answers and the points to draw out. Trainer reference.

WORKED EXAMPLE — one complete row to standard:

Root cause addressed: Untrained relief staff on the 07:00 roster (14-min vs 6-min draws).

Corrective action: Implement a structured 10-day phlebotomy onboarding with formal competency sign-off before any unsupervised draw.

Owner: Clinical Training Lead.

Timeline: Design by week 3; pilot at two centres weeks 4-8; full rollout by week 14.

Resources: S\$65,000 (trainer time, simulation arms, backfill cover); 0.4 FTE for 14 weeks.

Success measure: Mean relief-staff draw time from 14 min (baseline) to under 8 min by week 16; first-attempt success from 82% to 92%.

Key risk: Backfill cover unavailable during pilot, so centres refuse to release staff.

Mitigation: Clinical Director pre-commits agency budget for pilot weeks; escalation path named.

Review checkpoint: Week 8 — decision to roll out, extend the pilot, or stop.

KEY TEACHING POINTS:

1. THE OWNER IS THE PLAN'S SPINE. 'Operations' cannot be phoned, cannot be held to a date and cannot be asked why it slipped. A named role can. The Clinical Director's warning in the scenario is the most common real-world failure of improvement projects, and it is entirely preventable with one discipline: one named owner per action, no exceptions, no co-owners.
2. THE MEASURE MUST TIE TO THE BASELINE FROM TOPIC 1. This is where the course closes its own loop: the problem statement's baseline in Activity 1 is what makes evaluation possible in Topic 3. A team that skipped the baseline now literally cannot write this field — let them discover that themselves; it lands harder than being told.
3. MEASURABILITY CHECK: 'improved staff morale' is not measurable with data Meridian holds. 'Voluntary resignations per quarter' and 'early-shift roster fill rate' are. Force the substitution now, not at review time.
4. SEQUENCING: converting relief staff to permanent DEPENDS on the onboarding programme existing — convert first and you have permanent staff trained to the old inadequate standard, locking in the defect. Dependencies are where good plans quietly fail.
5. OWNER OVERLOAD: if the Centre Operations Manager owns three of four actions, the plan is a single point of failure regardless of how good each row looks. GenAI is genuinely useful at spotting this pattern mechanically — a good example of AI catching what a team misses because they are too close to it.
6. THE REVIEW CHECKPOINT MUST CARRY A DECISION. 'Review progress' is theatre. 'Decide to roll out, extend or stop' is governance.

Self-Check — Is Your Output Finished?

Your CAP is audit-ready when: all eight components are complete for all four actions; every owner is a named role and no role owns more than two actions; every success measure has a metric, a baseline number and a target number; every measure is obtainable from data the organisation already has (or has an explicit task to start collecting it); dependencies are sequenced; every mitigation is an action with an owner; and every checkpoint names a

decision. The real test: hand the plan to another team and ask them to execute row one without asking you a single question.